

To: Department of Health and Human Services (HHS)

From: The University of Kansas Health System

Subject: Response to RFI: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care (RIN 0955-AA13)

Executive Summary

The University of Kansas Health System is pleased to provide feedback on the HHS strategy for AI adoption. Our response is grounded in the "Principles for Responsible AI" developed by the College of Healthcare Information Management Executives (CHIME). We believe that for AI to improve patient outcomes and reduce clinician burden, the federal government must shift from a punitive regulatory culture to one that incentivizes innovation and shared responsibility.

I. Regulation: Moving from Penalties to Incentives

HHS should establish a regulatory environment that is predictable and proportionate to risk.

Incentive-Based Cybersecurity: We urge HHS to prioritize "carrots" over "sticks". Policies should reward providers for implementing recognized cybersecurity best practices rather than penalizing them when they are victims of sophisticated, AI-driven attacks.

National Privacy and AI Law: The current patchwork of state laws creates excessive administrative burden and costs. We support a comprehensive national privacy and AI law that preempts state requirements to ensure efficiency across the healthcare ecosystem.

Business Associate Agreement (BAA) Oversight: Many AI vendors use opaque contract practices or refuse to sign standard BAAs, shifting disproportionate liability to health systems. HHS should establish a standardized HIPAA BAA and increase oversight of Business Associates to ensure an equitable allocation of risk.

Stark and Anti-Kickback Statute (AKS) Modernization: Current laws deter larger health systems from donating cybersecurity and AI technology to smaller, under-resourced partners due to liability fears. HHS should update safe harbors to permit these vital donations, strengthening sector-wide resilience.

II. Reimbursement: Modernizing Payment for Innovation

HHS's payment policies must evolve to support high-value AI interventions.

Bridging the "Digital Divide": Small, rural, and under-resourced providers face insurmountable barriers to AI adoption, including infrastructure costs and lack of purchasing power. HHS should implement targeted funding, tax credits, and group purchasing options to ensure equitable access to AI tools.

Prevent Anti-Innovation Payer Terms: We are seeing a disturbing trend where payers include "anti-AI" clauses in their terms and conditions. These clauses ban providers from using AI or automation in revenue cycle processes. This creates a massive power imbalance, as many of these same payers use AI to automate their own claim denial processes. HHS should prohibit payers from contractually banning the use of responsible, provider-side AI that improves administrative efficiency.

Incentivizing Value over Volume: Modernize reimbursement to reward the use of AI tools that demonstrate long-term improvements in health outcomes and reductions in net spending.

III. Research & Development: Focusing on the Human Element

AI is a tool to augment, not replace, human clinical expertise.

Workforce Education and AI Literacy: HHS should invest in tailored education to help clinicians recognize AI limitations, such as "hallucinations" or algorithmic bias. Training must equip healthcare professionals to "trust but verify".

Standardized Model Cards: To reduce uncertainty, HHS should support the use of standardized "model cards". These should provide a transparent, regularly updated snapshot of a model's training data, performance metrics, and potential biases to facilitate informed purchasing decisions.

IV. Responses to Specific RFI Questions

Question 1: Biggest barriers to private sector innovation?

- Regulatory uncertainty, the "black box" nature of proprietary algorithms, and the high resource intensity required for continuous auditing are the primary hurdles.

Question 2: Regulatory or payment policy changes to prioritize?

- Prioritize Stark/AKS Safe Harbors: HHS should immediately update Stark and Anti-Kickback Statute (AKS) policies to clarify and expand cybersecurity and AI technology donation provisions.
- Rescind the proposed HIPAA Security Rule: The current proposal is duplicative and places a "crushing impact" on providers without meaningfully improving security.

- **Remove Duplicate CMS Mandates:** The Medicare Promoting Interoperability (PI) Program security risk management measure should be removed as it repeats requirements already found under HIPAA.

Question 3: Novel legal and implementation issues?

- **BAA Contract Overrides:** We are seeing vendors use main service agreements to override HIPAA Business Associate Agreement (BAA) restrictions, particularly regarding broad data use rights.
- **Liability Shifting:** AI vendors are increasingly capping their liability for data breaches at very low amounts (e.g., one year of service fees), which ignores the massive downstream risk to the health system and patients.
- **Contractual Asymmetry:** Some insurance companies are now adding language to their provider agreements that specifically prohibits the use of AI in revenue cycle management (RCM). This is a "bad faith" approach to contracting. It forces health systems to rely on manual, expensive, and slow processes while the payers use high-speed AI to find reasons to deny those same claims.
- **Inconsistent Definitions:** These "bans" often use broad, poorly defined terms for AI, which could inadvertently prohibit basic automation tools that have been industry standards for years. This creates a "chilling effect" on innovation.

Question 4: Most promising AI evaluation methods?

- We recommend the adoption of "Assurance Labs" and the use of standardized model cards that align with the "FAVES" framework (Fair, Appropriate, Valid, Effective, and Safe).

Question 5: How can HHS support private sector accreditation/testing?

- **Establish a "Better Business Bureau" for AI:** HHS should support a resource that lists AI solutions that have been determined to have met baseline security and safety standards.
- **Support for Small Providers:** HHS should provide funding and technical assistance to help under-resourced providers navigate the accreditation and auditing process, which is currently too resource-intensive for them.

Question 6: AI performance and potential?

- **Success in Ambient AI:** Ambient listening tools have shown significant success in reducing clinician documentation burden.
- **Potential for Chatbots:** AI-powered chatbots have high potential to reduce senior isolation and streamline administrative tasks like scheduling.

- Failures in ROI: Many AI tools have yet to deliver a justifiable return on investment (ROI) because they require massive infrastructure and governance that smaller systems lack.

Question 7: Key decision makers and hurdles?

- Decision Makers: AI adoption is driven by multidisciplinary governance boards, though clinical informatics and IT leadership play the primary role.
- Administrative Hurdles: The primary hurdles are the "black box" nature of models, which prevents clinician trust, and the time-consuming process of conducting individual risk assessments for every new tool.

Question 8: Where would enhanced interoperability widen market opportunities?

- Standardizing security requirements for third-party technologies and promoting interoperability between vendor security processes would reduce duplicative risk assessments and lower costs for providers

Question 9: Patient and caregiver concerns?

- Desire for Awareness, Not Overload: Patients want to know if AI is helping deliver their care, but they do not want to be overwhelmed with notifications for every minor background automation.
- Trust and Benefit: Patient comfort with AI tools like ambient documentation varies based on their level of trust in the provider and perceived benefit to their care.

Question 10: AI research priorities?

- Focus on Bias vs. Noise: Research should prioritize differentiating between systematic bias and random "noise" in clinical datasets.
- Impact on Clinical Skills: We need more research on the "tangible risk" that over-reliance on AI might lead to the deterioration of essential human clinical skills.